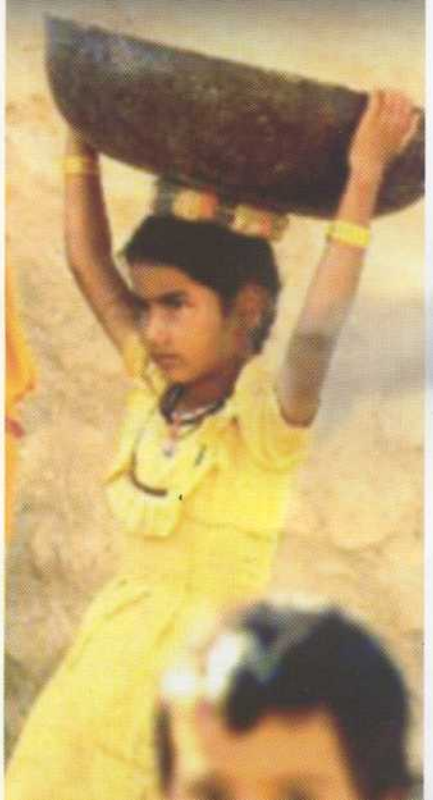


...LISTEN!

BHORUKA

BHORUKA WELFARE

The greatest challenge to India is the combination of her rapidly growing population and mass poverty. Poverty impedes social and economic development and increases inequalities in society. India contains an impoverished majority living traditional lives, seemingly untouched by the 21st century advancements. Government reforms have been hampered due to high rates of illiteracy and infant mortality, low status of women, conflict between castes and religious groups, inconsistent government policy and poor internal communication.





We need to 'care' to listen

How can poor people's voices be heard and represented among corporate citizens? How can we, as socially responsible individuals foster organizations to support capacity building and knowledge among the underprivileged? These are musings that have touched emotional chords at Bhoruka for forty years. The family tradition of reaching out to the underprivileged has been passed down from Bhoruka Group's Founder Father, PDji Agarwal who responded to 'the voices' in Rajasthan four decades ago. Today, Smt. Uma Agarwal and Shri. S N Agarwal take PDji's great ideals and noble initiatives forward through various organizations under a common foundation - Bhoruka Welfare.

The foundation is dedicated to helping the needy meet their potential through affirmative, sustainable action through various trusts - Bhoruka Charities, Ved Education Trust and Prabhudhan Rural Development Trust. The trusts are united in a commitment to help in the consistent development of initiatives aimed at constructively and efficiently aiding the underprivileged meet their potential, regardless of age, sex, caste or religion.

In a collaborative effort with SKDRDP (Shree Kshetra Dharmasthala Rural Development Project) promoted by Dr. D Veerendra Heggade, Bhoruka Charities supports "SIRI", a rural project that promotes small business ventures by Self Help Groups.

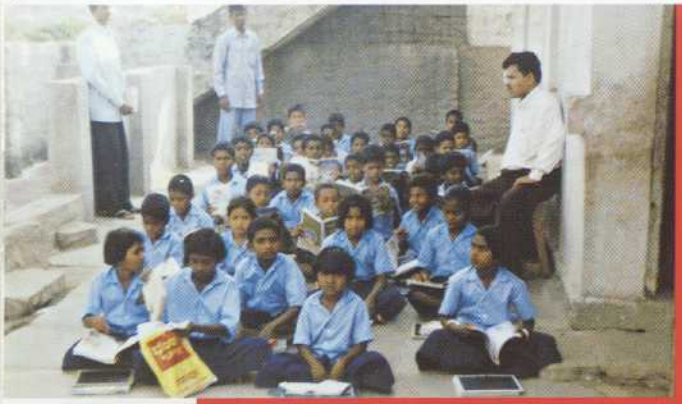
Bhoruka Welfare's social work has been extensive and penetrative, spanning Education, Healthcare, Community Development and Women Empowerment.

Today, as we carry the legacy forward, we perceive a bigger world wrought by greater exploitation. We understand the need for a bigger response. Statistics as provided by the Government of India show that about 240 million people live below the poverty line. Though there has been a steady decline in poverty over the last two decades, the total number of poor people has remained more or less constant due to growth in population. India's national goals of universal elementary education and total eradication of illiteracy still remain elusive. Women are much more likely than men to be poor, illiterate, neglected and abused.

The scenario sends a strong message to us. True, there are many more tears to be wiped...many more small dreams to be fulfilled...and many more doors to be opened. In essence, a lot remains to be done. In our effort to make a lasting difference to the community around us, we stand committed in carrying the mission of Bhoruka Welfare forward as PDji would have wanted us to.

EDUCATION

Children attend class in the Shahapur Residential Child Labour School



Bhoruka Composite Pre-University College, Bangalore



	INSTITUTIONS/SCHEMES	BENEFICIARIES	SUCCESS LEVELS
SCHOOLS	Bhoruka Composite Pre-University College, Blr. Bhoruka English School, Shahapur Bhoruka English School, Shivapur.	1,800	100%
ADOPTED SCHOOLS	Government Primary School, Rajankollur Government High School, Rajankollur Government Primary School, Hemmige Government Primary School, Neria	2,000	80%
CHILD LABOUR SCHOOL	Child Labour School, Shahapur	50	70%

44% of India's adult population is not literate. Child labour is rampant. Children are the backbone of the community's future intellectual capabilities and productive capacity, yet 80% of them do not have access to schools.

Literacy has a major role to play in undermining many social evils. Presently, at Bhoruka Welfare, spreading education is a huge objective. Extensive work has been accomplished in this area, many goals have been set and a substantial number of rural children have been brought under the various educational schemes promoted by the Welfare.

The Bhoruka Composite Pre-University College was established in 1971 on Tumkur Road, Bangalore, with classes from LKG to PUC. Today the school has a total strength of 900 students. Acknowledged as one of the best English medium schools in this region, the school has been receiving trophies from the Karnataka Government since 2001. The growth of the school has been exceptional. The students who've passed out of this school have done so with merit, winning awards and laurels for excellence in academics, sports and cultural talent. The education system is well rounded with qualified faculty and sound infrastructure to ensure the school's 100% success.

The School in Shahapur, Gulbarga District was established in 1996-97, and has classes from LKG to VII standard. The school has 500 students. Today, it has an ambitious target of expanding up to the XII standard through the addition of one standard every year. This school has become a much sought-after English medium school in the area.

The Residential Child Labour School was established in October 2002 at Shahapur. The prospect of earnings for rural children in urban areas



Children perform on Republic Day at Bhoruka English School, Shivapur



Bhoruka Adopted Government Middle School in Hemmige





Mass drill on Sports Day at Bhoruka School, Bangalore



High School students perform at a function at Bhoruka English School, Shivapur

India is home to 17% of the world's total population accommodated in an area which is 2.4% of the world's total area. Local dialects change in India almost after every 8-10 Kms. Educating this diverse mass is a big challenge.



forces underprivileged families to keep child labour alive in villages. Bhoruka steps in with the Residential Child Labour School where 35 children forming the first batch are being given quality education, nourishing food and uniforms. 50 children have been enrolled in the second batch and will be also attending school.

The School in Shivapur, Koppal District, was established in 1992. The School has classes from nursery to X standard and 500 students. This is the only English Medium School providing quality education within a radius of 10 kms in the area. This school has the distinction of having got excellent SSLC results by the very first batch of students. Since then the school has been consistently shining in academics, sports and cultural talent.

The Trust has adopted a Government Middle School in Hemmige Village in T. Narasipur Taluk. A new school building with five spacious rooms has been built at a cost of Rs. 5 lakhs. All amenities have been provided at the school with drinking water facility, toilets and a playground. The sound atmosphere in the school has helped the students achieve 100% results in the public examination. Orientation programmes have been held for the teaching staff regularly providing them with better teaching methods. This has improved the attendance of the students.

Under the education programmes in Dakshina Kannada district, the Trusts adopted the Government Middle School in Nidle. Three spacious classrooms have been built for the school at a cost of Rs. 3.5 lakhs. The facilities provided in the school have improved the attendance of the children, particularly girls. The trusts adopted a Government Middle and High School at Rajankollur in Gulbarga District at a cost of 4 lakhs.

COMMUNITY DEVELOPMENT

Tailoring classes being conducted for ladies at Hemmige



Ladies are being trained in Agarbathi-making



	INSTITUTIONS/SCHEMES	BENEFICIARIES	SUCCESS LEVELS
WOMEN EMPOWERMENT	Savings and credit programme Training on capacity building Exposure visits Bank linkage Revolving fund Smokeless chulas Dairy programme Sheep rearing and other micro enterprises	1,500	60%
YOUNG GIRLS	Skill training - Book binding, tailoring, knitting and embroidery, Agarbathi rolling, candle making and other skill training programmes. Self employment training (E.D.P)	250	65%
YOUNG MEN	Skill training - Radio & T.V. repair and service, electrical and home appliances, borewell repair, screen printing, book binding, etc..	150	62%
CHILDREN	Infant care (Healthcare) Enrolling dropout children to school Starting education centers in tribal settlements	2,500	71%
SHGs	125 SHGs	1,500	60%

India still has the world's largest number of poor people in a single country. Of its nearly 1 billion inhabitants, an estimated 350-400 million are below the poverty line, 75 per cent of them in the rural areas. More than 40 per cent of the population is illiterate, with women, tribal and scheduled castes particularly affected.

Ill health, malnutrition and high fertility are three main reasons why households become or remain poor. They cause poverty through diminishing productivity, reducing household income and increasing health expenditure. Today, Poverty reduction remains a pronounced challenge before Bhoruka. Extensive work has been already been done in this area.

In Bhoruka School premises - Bangalore, successful income-generation schemes have been introduced to train rural women in small industry skills, such as tailoring, embroidery, doll making and knitting, with the prospect of either helping them develop their own businesses or qualifying them for jobs in garment industries in the vicinity.

In Shahapur around 75 Self Help Groups have been formed in and around the village. Regular training is given in tailoring, book binding, TV and radio repair. Through the skills learned many of them have found employment and saved considerably out of their earnings. Over Rs. 10.5 lakhs were mobilized



Young men undergo training in repair of home appliances



Bhoruka-SKDRDP Centre for Rural Excellence at Belthangady



Most people in villages rely on the sun-parched land for sustenance. But barren soil and poor irrigation systems means crops are lean, locking families into an interminable cycle of poverty.

from 1000 ladies by the 75 SHGs and have been deposited in their respective bank accounts. To improve the civic amenities of Shahapur town, the main streets have been provided with lights to offer better visibility to commuters. Road dividers have been installed for safety.

The highly underdeveloped Mudbol Village in Shahapur Taluk was adopted under the Swasthi Gram Yojana, a scheme by the Karnataka Government. Proper roads were built, improved drainage systems were developed and general sanitation has been ensured in the village. A Government school building was also constructed.

In Hemmige to help pomote savings through self-employment and other income-generating activities, 50 Self Help Groups have been formed in 15 villages. The members meet every week and pay Rs. 10/- per head as part of their savings, which is monitored and controlled by the SHGs. Till date around Rs. 2,80,000 has been saved in banks and Rs. 50,000/- from the Bhoruka Group has been contributed as revolving funds to the groups. The income-generating activities in Hemmige, include, Agarbathi making, tailoring and embroidery courses for ladies and girls. Young men are trained in repair work in electrical and home appliances and have found employment in villages nearby.

SIRI

PRODUCTS	GROUPS
Fast moving chemical items	14
Condiments	11
Readymade garments	11
Agro processed	3
	39



SIRI is a collaborative effort between Bhoruka Welfare and Shri Kshetra Dharmasthala Rural Development Project (SKDRDP), to promote rural excellence. SKDRDP is guided by Padmabhushana Dr. Veerendra Heggade, who drives a mission to work with the poor and weaker sections, particularly with women who bear much of the abuse in backward communities. SIRI is one of the projects under this collaborative effort to empower women socially and economically by promoting their entrepreneurial skills. The SIRI center is located at Beltangady, Dakshina Kannada District, and is jointly built at a cost of Rs.37 lakhs. Under SIRI rural women are given training in lucrative business areas like dairying, composting, sericulture and bee keeping. The project also promotes business ventures like manufacturing chemical items, bakery products, rexine bags, readymade garments, squashes, pappads, condiments, sweets, wafers, pickle, etc.

Today the groups produce around 39 products. 35 groups involving 400 families are engaged in the production processes.

HEALTHCARE

Immunization of babies in Shahapur



Mobile dental check-up at Shivapur



	INSTITUTIONS/SCHEMES	BENEFICIARIES	SUCCESS LEVELS
WOMEN	(a) R.C.H. Programme - antenatal care, postnatal care. (b) General Health and Nutrition Awareness camps. (c) S.T.I./R.T.I. - HIV/AIDS awareness camps, iodine awareness camps. (d) Health checkup camps - eye camp, dental, cancer detection.	15,000	60%
CHILDREN	(a) R.C.H. Programmes - infant care, immunization (b) Baby shows, general checkup camps.	2,000	65%
MEN	(a) H.I.V/AIDS - Truckers community health programme. (b) General health checkup camps, eye camps dental camps and cancer detection camps. (c) Treatment for STD - STD Clinics.	10,000	62%

As per National AIDS Control Organization estimates, the total number of HIV infections in the country at the end of year 2000 stood at 3.86 million. In some states, the infection rate is one percent of the population.

Income poverty leads to ill health and ill health contributes to income poverty. Indian women have to pay much higher social and personal costs if they suffer from certain diseases. Besides poverty, the shame and stigma associated with the diseases, early marriage and social pressures to start a family early and limited access to treatment makes them more vulnerable to disease, more so during the reproductive age group of 15 - 45 years.

To reduce the burden of 'large familes' on the poorer sections, regular family planning camps are being conducted in Bhoruka School, Bangalore, for the last 20 years. Women undergo leproscopic operations every month, and the patients are given incentives to undergo these operations. Over 10,000 operations have been done till date.

In Shahapur the Trust has taken up Reproductive and Child Health (RCH) programme, a Central Government scheme in villages covering B.Gudi and Sagar PHCs in Gulbarga District. Adolescent Awareness Programmes and IEC activities to create general health awareness are conducted. Around 30,000 people from 25 villages have been brought under this scheme. Baby-health has also been ensured with immunization.

To improve the general health and well being of the people



Medical consultation for women under the RCH programme



Patients undergoing check-ups for cataract at the eye camp



The poor living conditions, low levels of education and absence of welfare provision, all contribute to the adverse health status of the majority of the population. Policies devised to improve this situation run the risk of being stifled within a system where corruption can dominate.

in the rural sector a number of health camps are organized regularly camps with certified doctors advise patients on eye, skin and dental care. General health check-ups are also conducted.

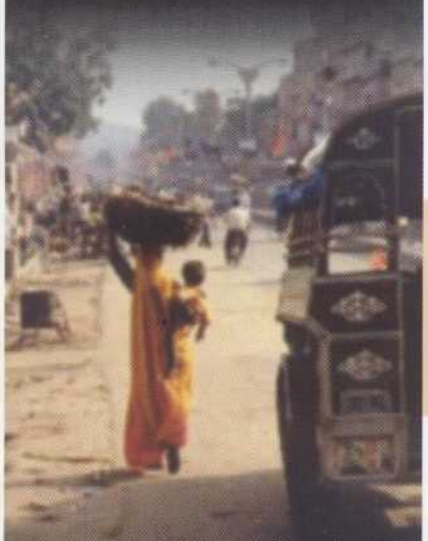
We are also committed to the promotion of the Indian System of Medicine and Homeopathy. Workshops help encourage practitioners use this system through locally available herbs, shrubs and roots. Every year four health melas are conducted and free ayurvedic medicines are given to patients. Around 2,000 patients are being helped each year since 1999. Farmers have been encouraged to grow medicinal plants to help the villagers to make use of this system.

The Truckers Community Health Programme, in Gulbarga and Wadi is introduced with the help of the Karnataka State Aids Prevention Society to create awareness and prevent HIV/AIDS among truckers. Over 2000 patients with sexually transmitted diseases have been treated each year since 1999. Around 10,000 truckers are covered under Behavior Change Communication and Condom Promotion. General health and awareness camps with IEC activities are regularly organized.

The Trust runs a health clinic at Shivapur to cater to the needs of the villagers as well as the Bhoruka Employees. A certified MBBS doctor and a trained nurse, with a social worker attend to around 30 to 40 out-patients every day. The clinic was set up in 1990 and ever since has been functioning with full support from the Trusts.

General check-up and awareness camps have been organized. A fully equipped mobile hospital conducting dental check-ups has been mobilized. Patients with eye problems have been identified and helped with cataract operations.

The struggle for basic amenities is ongoing. Bhoruka's approach is one of continual identifying of areas of social want. Our constant touch with the underprivileged keeps us aware of the burning issues that fuel neglect and deprivation. Increasing stress on education, empowerment of women and the economically weaker sections of society, will be our focus in helping the poor.



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BHORUKA

BHORUKA WELFARE